CASE STUDY 04 / 05

Medical Records & Coding Quality

Outpatient abstraction • ICD-10-CM review • privacy-first documentation

BUSINESS SCENARIO

A fictional outpatient clinic supplies six synthetic encounter records. Four include supported diagnoses or symptoms, while two require documentation or diagnostic holds. The reviewer must avoid speculative coding and preserve traceable QA and privacy decisions.

COMPLETED DELIVERABLE De-identified-style synthetic case notes, diagnosis abstraction register, coding hold log, documentation-completeness checklist, privacy and release controls.

ROLE COVERAGE

Health Information Technician • Medical Coding Specialist • Word Processing Specialist

EVIDENCE AT A GLANCE

6	4	2
SAMPLE ENCOUNTERS	CODED RECORDS	CODING HOLD

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Portfolio disclosure: All organizations, records, people, and transactions are fictional. This is a simulated work product, not a claim of past paid employment or client work.

01 / CODING SCOPE & CONTROLS

Classification standard and case boundaries

Attribute	Working requirement
Encounter setting	Outpatient ambulatory care; fictional clinical training cases
Service dates	2026-10-22 through 2026-10-26
Code version	FY 2027 ICD-10-CM, effective October 1, 2026–September 30, 2027
Code source	CDC / CMS official FY 2027 ICD-10-CM tabular list and guidelines
Scope limitation	Diagnosis abstraction only. No CPT/HCPCS procedure coding, DRGs, reimbursement or claims submission.
Privacy practice	Use synthetic encounter IDs; avoid unneeded identifiers; access limited by authorized role

Outpatient abstraction decision rule

- Use the final authenticated provider assessment; do not code a question-mark or ruled-out diagnosis as established in outpatient care.
- When documentation cannot support code selection or is unsigned, flag the account and request clarification; do not guess.
- Confirm code validity and instructions against the FY 2027 tabular list before actual use; code choices here are illustrative.
- Keep record completion, coding status, and disclosure authorization as distinct control decisions.

TRAINING-ONLY SAFEGUARD

All encounters and clinicians are fictional; no real patient data appears. This is a coding-workflow demonstration, not clinical advice, an authorization to bill, a credential, or a production coding determination.

02 / ENCOUNTER EVIDENCE

Synthetic clinical record excerpts

PX-101 — outpatient established visit / 2026-10-22

Chief reason: scheduled hypertension follow-up. Provider: Dr. Avery Lane. Final signed assessment: (1) Essential (primary) hypertension, controlled on the existing plan; (2) Type 2 diabetes mellitus without documented complications, monitored. Plan documents blood pressure review, home readings, follow-up interval, and continued metabolic monitoring. Signed 2026-10-22 16:20 ET.

PX-102 — outpatient sick visit / 2026-10-23

Chief reason: sore throat, congestion, and cough for three days. Provider: Dr. Devon Ellis. Final signed assessment: Acute upper respiratory infection, unspecified. The note documents supportive care instructions and return precautions. No confirmed secondary condition in the final assessment. Signed 2026-10-23 15:10 ET.

PX-103 — outpatient follow-up / 2026-10-24

Chief reason: tingling in both feet. Draft note: “diabetic neuropathy?” in an unsigned assessment. No finalized provider diagnosis, no authenticated signature, and no completed plan of care. Record received in incomplete state on 2026-10-27.

Completeness screening

Record	Assessment signed	Diagnosis support	Next stage
PX-101	Yes	Two established conditions	Coding review
PX-102	Yes	One established condition	Coding review
PX-103	No	Questioned diagnosis only	HOLD: provider query

03 / COMPLETED CODING ABSTRACTION

Preliminary ICD-10-CM diagnosis register

Visit	Sequence	Code	Documented condition	Disposition
PX-101	1st	I10	Essential (primary) hypertension	Supported
PX-101	2nd	E11.9	Type 2 diabetes mellitus without complications	Supported
PX-102	1st	J06.9	Acute upper respiratory infection, unspecified	Supported
PX-103	—	Not assigned	Unconfirmed “diabetic neuropathy?” and unsigned note	Held

Abstraction rationale

01 PX-101: hypertension is the stated chief reason and is managed; the signed assessment also documents uncomplicated type 2 diabetes. Reportable conditions are supported by the provider note.

02 PX-102: use the confirmed acute respiratory-infection diagnosis rather than separately coding symptoms without evidence they warrant separate reporting.

03 PX-103: the uncertain text is not a confirmed outpatient diagnosis and the note is unsigned. Withhold code selection until an authenticated final assessment is available.

RELEASE STATE

PX-101 and PX-102: coding review complete for training sample; actual claim release would still require payer/provider controls. PX-103: not ready; no code assigned.

04 / DEFICIENCY & CLARIFICATION LOG

Clinical documentation exceptions

ID	Record	Deficiency	Owner / request	State
HIM-01	PX-103	Unsigned clinical assessment	Route to treating provider for completion and authentication	Open
HIM-02	PX-103	Questioned diagnosis not established	Request final, clinically supported assessment; no leading code suggestion	Open
HIM-03	PX-101	No deficiency identified in sample	Routine completeness pass	Closed
HIM-04	PX-102	No deficiency identified in sample	Routine completeness pass	Closed

Provider clarification request (sample)

NON-LEADING QUERY

Encounter PX-103 is pending authentication. The draft assessment contains an unresolved question-mark diagnosis. Please complete and authenticate the encounter assessment, documenting the clinical condition(s) supported by your evaluation or confirming that no specific diagnosis has yet been established. The coding team will await the signed record.

Release-of-information routing example

Request type	Minimum documentation / handling
Patient access request	Verify identity and route through approved patient-access workflow; track receipt and response dates.
External third-party request	Check valid authority/authorization and applicable exceptions before release; route to privacy office.
Internal coding access	Role-based access to records required for coding and quality review.
Unclear or excessive scope	Escalate to privacy officer rather than disclose unverified records.

05 / QA, POLICY & REFERENCES

Medical record QA checklist

Sample completion controls

PASS 3/3 encounters in the original batch assigned unique fictional IDs and dates (6/6 including the evidence-edition extension).

PASS 2/2 coded encounters in the original batch contain final signed assessments (4/4 in the extended batch).

PASS 3 original coded diagnoses trace to signed assessments (5 in the complete extended register).

PASS One unsigned outpatient encounter remained held; the evidence edition adds a second diagnostic-uncertainty hold.

PASS No real patient names, dates of birth, addresses, insurance IDs, or actual PHI used.

Production-workflow guardrails

- Validate each code against the encounter-date code set, Alphabetic Index, Tabular List notes, and official coding guidelines before production use.
- Code assignment is a distinct activity from record completion, authorization to disclose, and claim submission.
- Apply organization-specific minimum-necessary policies where the rule is applicable, acknowledging that HIPAA has exceptions and operational nuances.
- Use formally approved EHR, encoder, release-of-information, and audit systems for live records.

Authoritative source register

CMS, FY 2027 ICD-10-CM Official Guidelines: https://www.cms.gov/files/document/fy-2027-icd-10-cm-coding-guidelines.pdf

CDC, ICD-10-CM official files: https://www.cdc.gov/nchs/icd/icd-10-cm/files.html

HHS, minimum necessary guidance: https://www.hhs.gov/hipaa/for-professionals/privacy/guidance/minimum-necessary-requirement/index.html

CONTROLLED RELEASE

Control	Final result
Document owner	Health Information Integrity / Coding QA
Review state	Synthetic abstraction reviewed; PX-103 remains on hold; no billing release
Review date	2026-10-27
Data classification	Fictional sample / no real personal, financial, or medical data

06 / EXTENDED CASE MIX

Six-encounter outpatient coding review

The original three illustrative cases are retained. Three new synthetic scenarios extend case-mix coverage, including a supported pain symptom, documented hyperglycemia and a second uncertainty hold. Source extracts and coded/held decisions are available as CSV to support review.

Encounter	Clinical evidence	Code / decision	Reason
PX-104	Signed: right knee pain evaluated	M25.561	Pain is supported; no definitive cause claimed
PX-105	Signed: type 2 DM with hyperglycemia	E11.65	Provider expressly documented hyperglycemia
PX-106	Signed: possible cystitis vs vaginitis	HOLD / no code	Uncertain outpatient diagnosis; more documentation needed

Training-case controls

Audit test	Result
Source encounters	6 fictional IDs, no patient identifiers
Final assessments with confirmed conditions	PX-101, PX-102, PX-104, PX-105
Unresolved documentation / diagnostic questions	PX-103 and PX-106 require human review
Known codes assigned in training register	5 diagnosis-code entries across 4 encounters
Billing / claims submission	None / explicitly excluded

07 / CODING DECISION QUALITY

Provider query, code validation and privacy safeguards

Non-leading follow-up example for PX-106

Please review and finalize the assessment for encounter PX-106. The signed note lists alternative possible conditions but does not identify a confirmed diagnosis. Document the condition(s) established at this encounter, or the symptoms/findings evaluated, and complete any appropriate follow-up. The coding team will assign codes only from supported final documentation.

Medical coder production-readiness checklist

- Check encounter date and coding system; use FY 2027 ICD-10-CM for October 2026 services.
- Start with the provider note and complete record; then check official Alphabetic Index and Tabular List.
- Verify Includes, Excludes, code-first/use-additional-code instructions and sequencing in full context.
- Treat suspected diagnoses differently by setting: outpatient suspected diagnoses are not reported as established.
- Do not assume that an authenticated note is automatically a claim-release approval.
- No patient identities, payer IDs, insurance numbers or real protected data are included.

Reference: CMS FY 2027 Official Guidelines for Coding and Reporting, including Section IV outpatient reporting rules; CDC official FY 2027 code files. These are deliberately simple exercises, not proof of specialty medical coding proficiency or credentials.

DEEP-DIVE CASE FILE / HIM-01

Expanded HIM quality assurance: 12 encounters

The original six synthetic outpatient encounters are retained. Six additional encounters test authentication, outpatient uncertainty, laterality, encounter-purpose confirmation, and documentation clarification. Codes are intentionally NOT invented for the new cases; a credentialed coder must verify the official FY 2027 code set and conventions before a real release.

Case	Document status	Release stage	Reason
PX-107	Signed	Code review - symptom only	Do not infer disease cause
PX-108	Unsigned	HOLD - unsigned	Obtain authenticated note
PX-109	Signed	Code review - history context	Review documentation and official index
PX-110	Signed	HOLD - uncertain diagnosis	Do not code suspected as confirmed outpatient diagnosis
PX-111	Signed	HOLD - specificity	Seek non-leading clarification
PX-112	Signed	Code review - verify purpose	Check official code set and encounter facts

Version and environment control

The FY 2027 ICD-10-CM code set applies to eligible encounters dated October 1, 2026 through September 30, 2027. ICD-10-CM Tabular List and Alphabetic Index instructions take priority over guideline summaries. A real coder must review the complete documented encounter, code-selection notes and payer requirements. The training package is not a coding credential or a billable coding product.

Official sources: https://www.cdc.gov/nchs/icd/icd-10-cm/files.html and https://www.cms.gov/files/document/fy-2027-icd-10-cm-coding-guidelines.pdf

DEEP-DIVE CASE FILE / HIM-02

Patient-information handling and neutral provider queries

Control	Pass criterion	Example hold
Authentication	Signed provider assessment available	PX-108 unsigned documentation
Specificity	Clinically documented detail supports selected code	PX-111 missing laterality
Diagnostic certainty	Do not treat “possible” as confirmed outpatient diagnosis	PX-110 possible infection
Record minimum necessary	Only approved roles access record data	No real patient data in training package
Release of information	Verify authority, scope and appropriate method	Document request ownership
Audit trail	Reviewer, version and reason are recorded	No untracked code edits

Non-leading query template

Case: PX-111. Subject: Documentation clarification - site/laterality. “The assessment describes wound follow-up but does not specify the documented side. Please clarify the clinical site and laterality based on your professional judgment, if known, or indicate that it cannot be determined.” Do not offer a favored diagnosis as a required answer.

Coding verification gate

- Confirm care setting (outpatient vs inpatient) and the effective code-set year.
- Read complete authenticated record, including relevant history, assessment, procedures and orders.
- Identify documented diagnosis or appropriate symptom/reason-for-visit only.
- Research Alphabetic Index followed by Tabular List; read inclusion/exclusion and sequencing instructions.
- Check documentation completeness, payer policies, and source traceability.
- Log uncertain or unsupported entries as hold; never fabricate definitive diagnosis.
- Complete independent QA, release status, version and retention controls.

DEEP-DIVE CASE FILE / HIM-03

High-risk defect scenarios and audit scoring

Failure mode	Why risky	Safeguard
False positive diagnosis	Speculative outpatient diagnosis treated as confirmed	Hold pending signed definitive note
Wrong code year	Code not valid on encounter date	Date-aware release validation
Missing laterality	Specificity unsupported	Neutral provider query
Unclear copy/paste	Conflicting assessment evidence	Discrepancy log; confirm source
Wrong recipient	Unauthorized PHI disclosure	Requestor identity / access check
Unredacted export	Sensitive content exposed	True redaction; search-text test
Unsigned note	Authentication uncertainty	HOLD; no clinical inference

QA sampling design

For this 12-case learning batch, each encounter is reviewed against six gates: authenticity, setting, documented problem, specificity, code-year readiness, and release status (72 pass/hold observations). No coding-accuracy percentage is claimed because six new exercises have deliberately unassigned codes and have not been independently reviewed by a certified coder.

Communication and confidentiality

All patient labels and notes are fictional. Never import actual patient details into portfolio materials or public AI tools without approved authorization. PDF demonstrations in this package use invented names and true text redaction; no real PHI is involved.